E/M UNIVERSITY

E/M Case of the Week

This service should be coded as a level 2 progress note plus one hour of prolonged inpatient services.

99232 and 99356

$64.76 + 90.12 = $154.88

E/M	History	Exam	MDM	Time
99231	PF	PF	SF/Low	15
99232	EPF	EPF	Mod	25
99233	Det	Det	High	35
(Requires 2 out of 3 key components)				

THIS SHOULD BE A LEVEL 3 IF BETTER PHYSICAL EXAM DOCUMENTED

CHIEF COMPLAINT: Chest pain.

INTERVAL HISTORY: The patient's chest pain has resolved.

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/3
Comp	Ext	≥ 10	3/3

REVIEW of SYSTEMS
Cardiovascular: Negative for DOE, PND.

PHYSICAL EXAM
CONSTITUTIONAL: Conversant; NAD.
Vitals: BP 138/75, HR 84, RR 22.
CARDIOVASCULAR: RRR, no MRGs.
RESPIRATORY: CTA.

COULD DO A BETTER PHYSICAL EXAM

Exam	Bullets Required
PF	1 - 5 from any organ systems
EPF	6 - 11 from any organ systems
Detailed	≥ 12 from any organ systems
Comp	2 bullets from NINE systems

LABS/DATA: Reviewed. Troponins remain in the normal range. LDL 130. Nuclear stress test reported as possible infero-lateral ischemia.

1 POINT FOR LABS, 1 POINT FOR STRESS TEST

IMPRESSION:

COULD GET 2 POINTS IF YOU DISCUSSED STRESS TEST WITH CARDIOLOGIST

1. Resolving typical chest pain, most likely ischemic. 1 POINT
2. Newly diagnosed hyperlipidemia. 3 POINTS

PLAN:

1. Start SIMVASTATIN 10 mg PO QD.
2. I educated the patient about his positive stress test.
3. It is my opinion that he should undergo cardiac catheterization.
4. The patient requests that I come back later and discuss these issues when his wife is present.

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

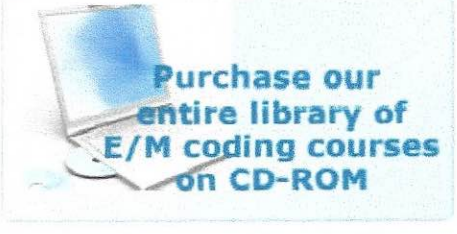
ADDENDUM: I came back to see the patient and his wife to discuss the risks and benefits of cardiac catheterization in the current clinical circumstances. All questions were answered to their satisfaction. He and his wife have tentatively decided on cardiac catheterization in the morning.

Total time spent today in the course of two separate visits was 56 minutes.
(Should document start and stop time of second visit)

Problem Points 4
Data Points 2
Risk

CARDIAC CATH IS CONSIDERED HIGH RISK, I WOULD CONSIDER HIGH RISK

REVISED
8:02 pm, Nov 18, 2008



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E/M UNIVERSITY

Case of the Week

By Peter R. Jensen, MD, CPC

The E/M service documented is a level three hospital progress note.

99233

98.13

E/M	History	Exam	MDM	Time
99231	PF	PF	SF/Low	15
99232	EPF	EPF	Mod	25
99233	Det	Det	High	35
(Requires 2 out of 3 key components)				

CHIEF COMPLAINT: CHF

INTERVAL HISTORY: CHF symptoms worsened since yesterday. Now has some resting dyspnea. HTN remains poorly controlled with systolic pressure running in the 160s. Also, I'm concerned about his CKD, which has worsened, most likely due to cardio-renal syndrome.

THREE CHRONIC PROBLEMS

REVIEW OF SYSTEMS: Positive for orthopnea and one episode of PND. Negative for flank pain, obstructive symptoms or documented exposure to nephrotoxins.

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	X
Comp	Ext	≥ 10	3/3

PHYSICAL EXAMINATION:
GENERAL: Mild respiratory distress at rest
VITAL SIGNS: BP 168/84, HR 58, temperature 98.1.
LUNGS: Worsening bibasilar crackles
CARDIOVASCULAR: RRR, no MRGs.
EXTREMITIES: Show worsening lower extremity edema.

Exam	Bullets Required
PF	1 - 5 from any organ systems
EPF	6 - 11 from any organ systems
Detailed	≥ 12 from any organ systems
Comp	2 bullets from NINE systems

LABORATORY DATA: BUN 56, creatinine 2.1, K 5.2, HGB 12. 1 POINT

IMPRESSION:
1. ~~Severe exacerbation of~~ CHF 2 POINTS
2. Poorly controlled HTN 2 POINTS
3. Worsening acute kidney injury due to cardio-renal syndrome 2 POINTS

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

PLAN:
1. Increase BUMEX to 2 mg IV Q6.
2. Give 500 mg IV DIURIL times one.
3. Re-check usual labs in a.m.

HIGH RISK BIC OF CHF EXACERBATION / POORLY CONTROLLED HTN AND WORSENING RENAL FAILURE

Total time: 20minutes.

Problem Points	6
Data Points	1
Risk	HIGH RISK

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Case of the Week

By Peter R. Jensen, MD, CPC

The E/M service documented is a level THREE hospital progress note.

99233

$98.66

E/M	History	Exam	MDM	Time
99231	PF	PF	SF/Low	15
99232	EPF	EPF	Mod	25
99233	Det	Det	High	35
(Requires 2 out of 3 key components)				

A hospital progress note for a patient with new AFIB...

CC: COPD exacerbation

INTERVAL HISTORY: The patient's COPD exacerbation has improved with nebs and steroids. HTN is adequately controlled with systolic blood pressures in the 130s. CAD remains stable with no active chest pain. The patient complains of intermittent palpitations.

THREE CHRONIC PROBLEMS WERE COMMENTED ON

ROS: CV: negative for CP/orthopnea. Pulmonary: negative for cough, improving SOBAR.

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	X
Comp	Ext	≥ 10	3/3

PHYSICAL EXAM: BP is 138/66, HR 102, RR 18. NAD, conversant. Lungs have scattered wheezes. Heart is irregular, no MRGs. Abdomen is soft, non-tender. Extremities show no peripheral edema.

Exam	Bullets Required
PF	1 - 5 from any organ systems
EPF	6 - 11 from any organ systems
Detailed	≥ 12 from any organ systems
Comp	2 bullets from NINE systems

LABS: BUN 12, creatinine 0.8, HGB 14 LABS 1 POINT

EKG was reviewed and shows new AFIB with no ST changes 2 POINTS

IMPRESSION

1. New onset AFIB 4 POINTS
2. Improving COPD exacerbation 1 POINT
3. Stable CAD 1 POINT
4. Well controlled HTN 1 POINT

Medical abbreviations used in this note

PLAN

1. Serial troponins.
2. Start heparin drip.
3. Nuclear stress test in a.m. – 1 POINT FOR ORDERING NM STRESS
4. Consult cardiology.
5. Convert to oral prednisone.
6. Change nebs to PRN.

Problem Points 7

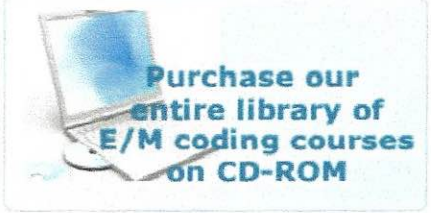
Data Points 4

Risk

HIGH RISK → ON HEPARIN DRIP AND NEW ONSET AFIB

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			





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Case of the Week

By Peter R. Jensen, MD, CPC

The E/M service documented is a level THREE hospital progress note.

99233

$98.13

E/M	History	Exam	MDM	Time
99231		PF	SF/Low	15
99232		EPF	Mod	25
99233		Det	High	35
(Requires 2 out of 3 key components)				

CC: F/U CHF

INTERVAL HISTORY: Feels a little worse today.

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	1	None
Detailed	[illegible]	2-9	1/3
Comp	Ext	≥10	3/3

Vitals: 160/90, 18, 82, 98.6
General: NAD, well-nourished, looks stated age
Neck: FROM, trachea midline; no JVD
Lungs: Bibasilar crackles; normal respiratory effort
CV: RRR, no MRGs, normal PMI in the MCL
Abdomen: Soft, non-tender; no HSM
Ext: 2+ peripheral edema; no digital cyanosis
Skin: Warm and dry; well-perfused

BNP 1450

138	101	10 / 124	12
3.1	28	0.8	36

Exam	Bullets Required
PF	1 - 5 from any organ systems
EPF	6 - 11 from any organ systems
Detailed	≥ 12 from any organ systems
Comp	2 bullets from NINE systems

Assessment

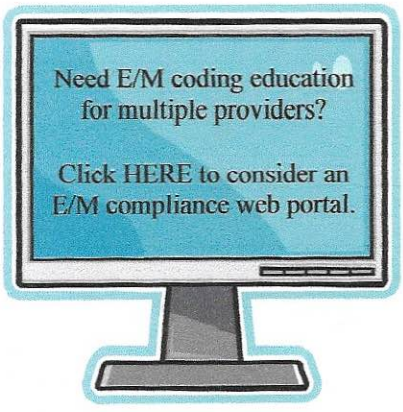
1. Severe, worsening CHF exacerbation 2 POINTS
2. Poorly controlled HTN 2 POINTS
3. Mild hypokalemia 1 POINT

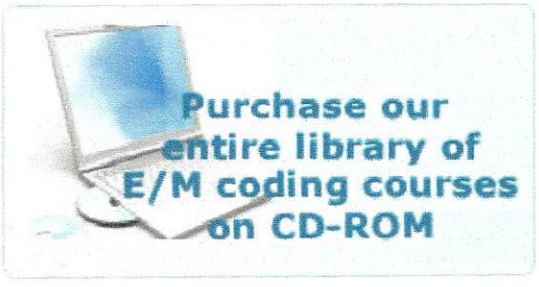
Plan

1. D/C PO LASIX
2. Start IV BUMEX 2 mg Q 6H
3. Replete potassium per protocol
4. Renal profile and BNP in a.m.

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

Problem Points	5
Data Points	1
Risk	HIGH RISK BECAUSE of CHF EXAC





On-Site Physician-to-Physician E/M Coding Education



Peter R. Jensen, MD, CPC

E/M UNIVERSITY

Case of the Week

By Peter R. Jensen, MD, CPC

The E/M service documented is a level THREE hospital progress note.

99233

$92.94

E/M	History	Exam	MDM	Time
99231	PF	PF	SF/Low	15
99232	EPF	EPF	Mod	25
99233	Det	Det	High	35
(Requires 2 out of 3 key components)				

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/3
Comp	Ext	≥ 10	3/3

Exam	Bullets Required
PF	1 - 5 from any organ systems
EPF	6 - 11 from any organ systems
Detailed	≥ 12 from any organ systems
Comp	2 bullets from NINE systems

CC: HTN

INTERVAL HISTORY: The patient continues to have severe intermittent, labile HTN which has been associated with pounding headaches.

QUALITY → LABILE

Review of systems is negative for chest pain or palpitations. Positive for occasional blurry vision.

Current medications: LISINOPRIL 40 mg PO QD, ATENOLOL,100 mg PO QD, LASIX 40 mg PO QD, AMLODIPINE 10 mg PO QD, DOXAZOSIN 8 mg PO QD. LISTING MEDS COUNTS AS ONE ELEMENT OF PAST MEDICAL HISTORY

PHYSICAL EXAM: NAD, 190/100, 72, 24. Lungs CTA. Heart RRR with no MRGs. No peripheral edema.

Labs: BUN 23, creatinine 2.2, HGB 11.8

1 POINT

Impression

1. Severe HTN which is not yet controlled. 2 POINTS
2. Worsening acute kidney injury. 2 POINTS

Problem Points	4
Data Points	1
Risk	

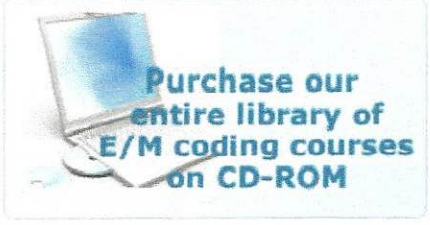
HIGH RISK GIVEN WORSENING HTN + RENAL FAILURE

PLAN

1. Add MINOXIDIL 5 mg PO BID.
2. Start PRN IV LABETALOL for SBP > 190.
3. Check renal artery Doppler study to rule out RAS.
4. Repeat renal profile in a.m.

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			





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Case of the Week

By Peter R. Jensen, MD, CPC

The E/M service documented is a level THREE hospital progress note.

99233

$95.85

E/M	History	Exam	MDM	Time
99231	PF	PF	SF/Low	15
99232	EPF	EPF	Mod	25
99233	Det	Det	High	35
(Requires 2 out of 3 key components)				

IF YOU DID DETAILED EXAM, YOU DON'T NEED HIGH MDM TO QUALIFY FOR LEVEL 3

CC: F/U Pancreatitis

INTERVAL HISTORY: Patient continues to have constant gnawing mid-epigastric abdominal pain which is 8/10 in severity.

ROS: Positive for intermittent nausea and vomiting. Negative for melena or hematemesis. No chest pain or shortness of breath.

PHYSICAL EXAM: BP 128/82, HR 72, RR 25. Anicteric sclerae. Lungs are CTA. Heart RRR with no MRGs. Abdomen is soft with diffuse tenderness. No peripheral edema.

Labs: WBC 13,000, HGB 9.4, PLT 232,000. BUN 36, Creatinine 2.1, Calcium 10.1, Potassium 4.2, Lipase 322. 1 POINT

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	✗
Comp	Ext	≥ 10	3/3

Exam	Bullets Required
PF	1 - 5 from any organ systems
EPF	6 - 11 from any organ systems
Detailed	≥ 12 from any organ systems
Comp	2 bullets from NINE systems

IMPRESSION
1. Worsening pancreatitis and abdominal pain 2 POINTS
2. New finding of ARF, most likely pre-renal 3 POINTS
3. Improving anemia 1 POINT

PLAN
1. MEPERIDINE 75 mg IV now then Q4H, PRN
2. Renal profile and CBC in a.m.
3. Continue NPO status

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

Problem Points	6
Data Points	1
Risk	HIGH RISK BECAUSE OF IV MEPERIDINE AND ACUTE RENAL FAILURE

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Case of the Week

By Peter R. Jensen, MD, CPC

The E/M service documented is a level three hospital progress note.

99233

$98.13

E/M	History	Exam	MDM	Time
99231	PF	PF	SF/Low	15
99232	EPF	EPF	Mod	25
99233	Det	Det	High	35
(Requires 2 out of 3 key components)				

CC: ARF

INTERVAL HISTORY: Patient has severe (SEVERITY) but non-oliguric (QUALITY) ARF, which has worsened over the past two days (DURATION) and is now associated with moderate hyperkalemia (ASSOCIATED SIGNS/SX). Denies SOBAR or N/V.

History	HPI	ROS	PFSH
PF	Brief	None	
EPF	Brief	≥ 1	
Detailed	Ext	2 - 9	
Comp	Ext	≥ 10	

Exam: NAD, conversant. BP 118/80, RR 24, HR 98. NECK: No JVD. Lungs CTA. HEART: RRR, no MRGs; no peripheral edema.

Data: BUN 86, creatinine 3.6, K 5.6, HGB 8.7, PLT 168 1 POINT

Exam	1995 Exam Rules
PF	One body area or organ system
EPF	Two to four body areas and/or organ systems
Detailed	Five to seven body areas and/or organ systems
Comp	8 - 12 organ systems

Impression

1. Worsening ARF 2 POINTS
2. New moderate hyperkalemia 3 POINTS
3. Worsening anemia 2 POINTS

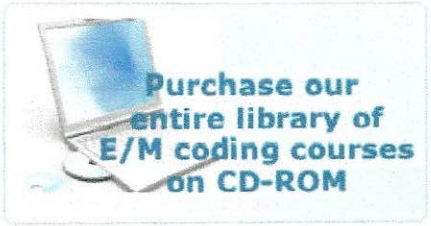
Plan

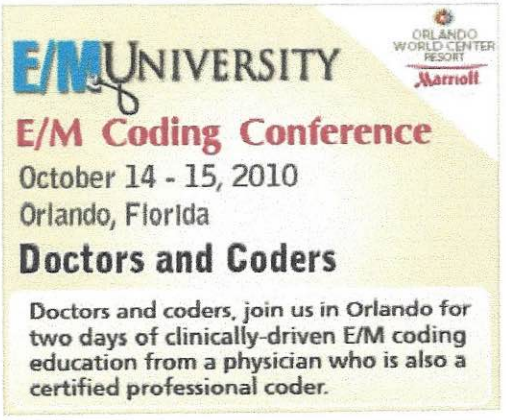
1. Transfuse 2 units PRBCs
2. Increase NS to 150 cc/hr
3. Low K diet
4. Repeat CBC and chem 7 in a.m.

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

Problem Points	7
Data Points	1
Risk	HIGH RISK GIVEN WORSENING RENAL FAILURE AND WORSENING ANEMIA







E/M UNIVERSITY

Case of the Week

By Peter R. Jensen, MD, CPC

The E/M service documented is ONE hour of critical care.

99291

$264.89

INTERVAL HISTORY: The patient was off the floor today when I rounded this morning. Saturation reported is in the 80s, but on my arrival on the floor on 6 L (saturation previously reported in the high 80s on 3 L) saturation in the 75% range on 6 L. The patient again relates his dyspnea to the onset of receiving Coumadin. He is now coughing up significant blood.

GENERAL: No distress. VITAL SIGNS: Saturation is 78% on 6 L. HEENT: No jaundice. Very loose dentures. NECK: No masses. HEART: Regular rhythm. No murmurs. LUNGS: Equal respiratory excursion. No use of accessory muscles. No wheezes, rales, or rhonchi. ABDOMEN: Nontender. EXTREMITIES: Without cyanosis, clubbing, or edema.

LABORATORY DATA: WBC 12.2, hemoglobin 10.5, and hematocrit 32. Westergren sedimentation rate is 71.Glucose 130, sodium 135, potassium 3.9, chloride 100, CO2 of 25, BUN 41, and creatinine 1.9. ANCA level is negative. Antiglomerular basement membrane antibody is pending. ARTERIAL BLOOD GASES: Arterial blood gases done stat on 5 L reveal pO2 of 42, pH of 7.47, and pCO2 of 31. The patient was placed on a nonrebreathing mask and saturation rose to 95.

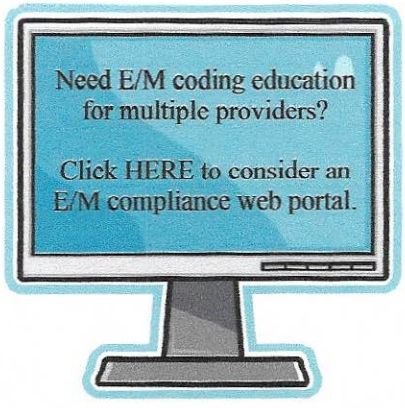
IMPRESSION:

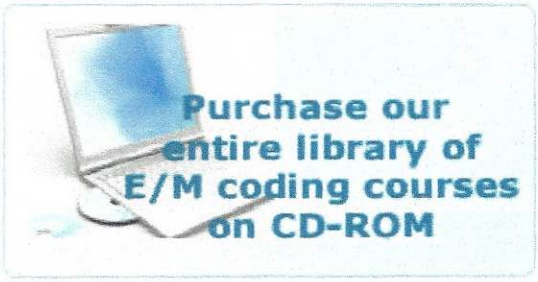
Bilateral infiltrates with presentation of hemoptysis, but rapidly progressive. Probably not bacterial at this point. I doubt pulmonary hemorrhage related to Coumadin, but cannot exclude. I suspect organizing pneumonia or rapidly progressive interstitial fibrosis (acute interstitial pneumonia). I doubt nonspecific interstitial pneumonia. Chest x-ray was relatively clear in January. I doubt acid-fast bacillus disease, but we will collect sputum. Amiodarone toxicity, atypical but possible.

PLAN:

1. For now, continue vancomycin and Zosyn.
2. IV vitamin K in the event this represents pulmonary hemorrhage.
3. Continue IV corticosteroids - would not stop. No objection into lowering as low as 60 mg q.12 h., but given the interstitial pneumonia, I would not stop these at the present time.
4. The patient must be off amiodarone.
5. I had a long discussion with the patient regarding life support. He has a limited concept of mechanical ventilation. I also explained bronchoscopy and lung biopsy. Initially told him if the physicians think he is terminally ill (he has no family), then we should not provide support. He is agreeable to a short term.
6. ICU transfer pending.

CRITICAL CARE TIME: 1 hour 10 minutes, delivered at the time of transfer to the Intensive Care Unit.





Answer Key: Case of the Week for 9/3/08

E/M UNIVERSITY

E/M Insight Case of the Week

In this case, the correct answer is that you would code for both the progress note (99233) plus one hour of critical care (99291).

$90.42 (99233)
$205.20 (99291)

I saw one of my patients in the hospital who was being treated for CHF. He had worsening kidney function and I thought we had over-diuresed him. I stopped diuretics and started him on IV fluids. I documented a level three hospital progress note (99233) at 08:30.

Then…the following note was documented:

CTSP stat for acute S.O.B

Earlier today, diuretics were discontinued due to worsening azotemia. The patient was started on conservative volume expansion with isotonic saline. Two hours later he developed chest pain and severe hypotension. Exam showed severe bibasilar crackles more than half way up. Troponin was 2.3. EKG showed ST depression in the lateral leads and peaked T waves diffusely. Stat chem. 7 showed creatinine of 3.2 with a potassium of 6.6. We gave one dose of kayexalate on the floor along with two grams of calcium gluconate and transferred the patient to the ICU.

Impression:
1. Acute MI with cardiogenic shock
2. ARF
3. Severe hyperkalemia
4. Underlying ischemic cardiomyopathy

Plan:
1. Start heparin
2. 10 units regular insulin
3. Two amps bicarb
4. Repeat chem. 7
5. Consult cardiology
6. Consult nephrology

Total critical care time on the floor and in the ICU: 45 minutes

For more information on coding for critical care, click HERE.

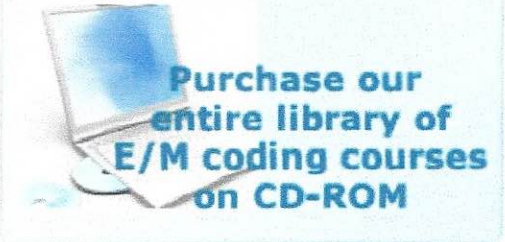
For information on the individual critical care codes click below:

- 99291
- 99292

E/M UNIVERSITY
E/M Insight Tips

Medical abbreviations used in this note

E/M UNIVERSITY
E/M Coding E-Learning for CME



E/M UNIVERSITY

Case of the Week

The E/M service documented is a level three hospital progress note.

99233

$92.94

E/M	History	Exam	MDM	Time
99231	PF	PF	SF/Low	15
99232	EPF	EPF	Mod	25
99233	Det	Det	High	35
(Requires 2 out of 3 key components)				

Hospital progress note for a patient admitted with CHF...

CC: F/U chest pain.

Interval History: The patient's CHF exacerbation has improved. However he now complains of intermittent substernal chestpain described as burning and associated with SOB. Denies cough. No nausea or vomiting.
TIMING, LOCATION, QUALITY, ASSOCIATED SIGNS/SX

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	X
Comp	Ext	≥ 10	3/3

PHYSICAL EXAM
CONSTITUTIONAL: NAD, conversant; look stated age
Vitals: BP 130/75, HR 84, RR 24
CARDIOVASCULAR: RRR, no MRGs, normal PMI.
RESPIRATORY: Lungs CTA; normal respiratory effort
GI: Abdomen soft, non-tender, no HSM
MUSCULOSKELETAL: No peripheral edema or digital cyanosis.

Exam	Bullets Required
PF	1 - 5 from any organ systems
EPF	6 - 11 from any organ systems
Detailed	≥ 12 from any organ systems
Comp	2 bullets from NINE systems

LABORATORY INFORMATION: Shows a BUN of 10 with a creatinine of 0.6, sodium 138, potassium 4.8, bicarbonate 24, calcium 8.3, PO4 3.9, and hemoglobin of 12.7. EKG was reviewed and showed no diagnostic ST changes. CXR report from this morning indicated improving pulmonary vascular congestion.
1 POINT FOR LABS
2 POINTS FOR REVIEWING EKG
1 POINT FOR CXR

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

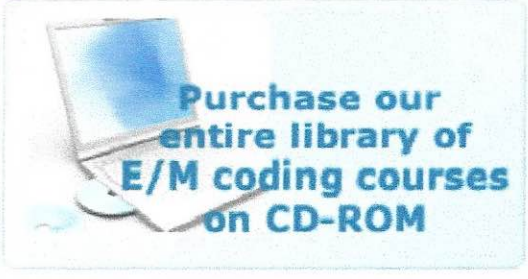
IMPRESSION:

1. New chest pain in patient admitted with CHF exacerbation. → 4 POINTS FURTHER WORKUP
2. Improving CHF. 1 POINT

Problem Points	5
Data Points	4
Risk	SHOULD BE CONSIDERED HIGH RISK BECAUSE OF CHEST PAIN

PLAN:

1. Serial troponins.
2. Sublingual NTG, prn.
3. Continue current loop diuretics.
4. Repeat renal profile in a.m.



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E/M UNIVERSITY

Case of the Week

By Peter R. Jensen, MD, CPC

The E/M service documented is a level THREE hospital progress note.

99233

$92.94

E/M	History	Exam	MDM	Time
99231	PF	PF	SF/Low	15
99232	EPF	EPF	Mod	25
99233	Det	Det	High	35
(Requires 2 out of 3 key components)				

CC: CVA and HTN

INTERVAL HISTORY: Patient remains intubated in the ICU following acute CVA with intracranial hemorrhage. No history can be obtained.
GET FULL CREDIT

Review of systems cannot be obtained due to the fact that the patient is intubated and sedated.

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/3
Comp	Ext	≥ 10	3/3

CONSTITUTIONAL: Intubated, sedated, unresponsive.
VITAL SIGNS190/100, 72, 24
NECK: No JVD, trachea midline
LUNGS: CTA with symmetric breath sounds
CARDIOVASCULAR: RRR, no MRGs, no peripheral edema
ABDOMEN: Soft, NABS, no HSM

Exam	1995 Exam Rules
PF	A limited exam of affected body area or organ system
EPF	Limited exam of affected organ system and other symptomatic or related organ systems
Detailed	Extended exam of the affected body area or organ systems and other symptomatic or related organ systems
Comp	8 - 12 organ systems

Labs: BUN 23, creatinine 1.2, HGB 11.8. ABG: PaO2 88/PaCO2 48 on 40% FiO2. 1 POINT FOR LABS

IMPRESSION

1. Severe HTN which is not yet controlled 2 POINTS
2. CVA with ICH 1 POINT
3. Respiratory failure 1 POINT

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

PLAN

1. Start NICARDIPINE drip
2. Titrate to SBP 140 mm Hg
3. Increase vent rate to 22
4. Continue tube feeds
5. Chem 10, CBC in a.m.

Problem Points	4
Data Points	1
Risk	

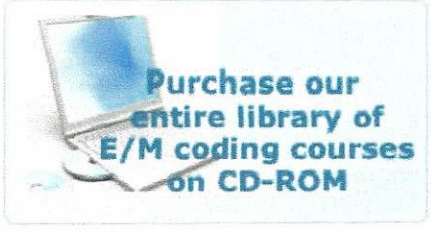
HIGH RISK FOR SEVERE HTN
CVA IS ALSO HIGH RISK → CHANGE IN NEURO STATUS

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Case of the Week

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99233

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E/M	History	Exam	MDM	Time
99231	PF	PF	SF/Low	15
99232	EPF	EPF	Mod	25
99233	Det	Det	High	35
(Requires 2 out of 3 key components)				

A Hospital Progress Note

CC: SOB

INTERVAL HISTORY: The patient's SOB has improved, but she now complains of intermittent chest pain described as a pressure-like sensation which is worse with exertion. TIMING, LOCATION, QUALITY, MODIFYING FACTOR

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	X
Comp	Ext	≥ 10	3/3

REVIEW OF SYSTEMS
CV: per interval history; negative for orthopnea/PND
GI: Negative for N/V

PHYSICAL EXAM
CONSTITUTIONAL: Conversant; NAD.
Vitals: BP 120/55, HR 84, RR 22.
CARDIOVASCULAR: RRR, no MRGs.
GI: Abdomen is soft, non-tender; no HSM.
RESPIRATORY: Improving bilateral wheezes.
MUSCULOSKELETAL: No edema or digital cyanosis.
SKIN: Warm and dry; well-perfused.

Exam	1995 Exam Rules
PF	A limited exam of affected body area or organ system
EPF	Limited exam of affected organ system and other symptomatic or related organ systems
Detailed	Extended exam of the affected body area or organ systems and other symptomatic or related organ systems
Comp	8 - 12 organ systems

LABS/DATA: BUN of 12 with a creatinine of .7, electrolytes stable; EKG was reviewed and showed NSR with no diagnostic ST changes.
1 POINT FOR LABS, 2 POINTS FOR EKG

IMPRESSION

Problem Points	5
Data Points	4
Risk	MODERATE RISK

1. Improving COPD exacerbation. 1 POINT
2. New chest pain--possibly cardiac. 4 POINTS

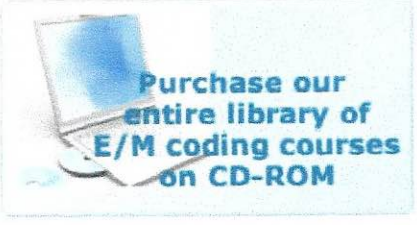
MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

PLAN

1. D/C SOLUMEDROL.
2. Start oral PREDNISONE.
3. Check stat troponins.
4. Will order CXR today to rule out pneumonia. 1 POINT FOR ORDERING CXR
5. Renal profile, CBC in a.m

E/M Pathfinder

Click here to translate your cognitve labor into the highest ethical level of care



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Case of the Week

By Peter R. Jensen, MD, CPC

The E/M service documented is a level THREE hospital progress note.

99233

$92.94

E/M	History	Exam	MDM	Time
99231	PF	PF	SF/Low	15
99232	EPF	EPF	Mod	25
99233	Det	Det	High	35
(Requires 2 out of 3 key components)				

CC: Shock

INTERVAL HISTORY: Patient admitted last night with altered mental status. Found to have acute renal failure. Intubated later for hypoxemic respiratory failure. Remains on LEVOPHED and DOPAMINE.

LINK

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/ 3
Comp	Ext	≥ 10	3/3

CONSTITUTIONAL: Intubated, unresponsive, cachectic
VITAL SIGNS: BP 80/60, HR 122, 24
NECK: No JVD, trachea midline
LUNGS: CTA with symmetric breath sounds
CARDIOVASCULAR: RRR, no MRGs, no peripheral edema
ABDOMEN: Soft, decreased bowel sounds, no HSM
MUSCULOSKELTAL: Severe muscle wasting and atrophy

Exam	1995 Exam Rules
PF	A limited exam of affected body area or organ system
EPF	Limited exam of affected organ system and other symptomatic or related organ systems
Detailed	Extended exam of the affected body area or organ systems and other symptomatic or related organ systems (5 - 7 systems?)
Comp	8 - 12 organ systems

Labs: BUN 285, creatinine 7, HGB 5.8. Urine creatinine 345, urine sodium 22. ABG: pH 7.13, PaO2 128, PaCO2 40 on 40% FiO2. 1 POINT

IMPRESSION

1. Severe, but improving ARF due to profound volume depletion 1 POINT
2. Worsening anemia 2 POINTS
3. Worsening acidosis 2 POINTS
4. Hypovolemic shock, not yet improving 2 POINTS

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High
Requires 2/3 dimensions			

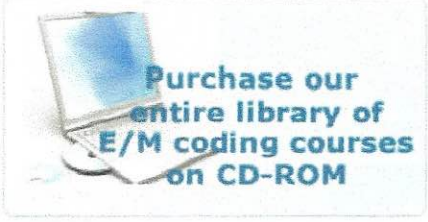
PLAN

1. Another liter NS wide open
2. Then isotonic bicarb at 300 cc/hr
3. Transfuse 2 units PRBCs
4. Repeat chem. 7, CBC at 1400
5. Usual labs in a.m.

Problem Points	5
Data Points	1
Risk	HIGH RISK GIVEN SHOCK

Total critical care time: 25 minutes





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Case of the Week

By Peter R. Jensen, MD, CPC

2/24/09

The E/M service documented is a level THREE Admission H&P

99223

$193.21

E/M	History	Exam	MDM	Time
99221	Det	Det	SF/Low	30
99222	Comp	Comp	Mod	50
99223	Comp	Comp	High	70

(Requires 3 out of 3 key components)

History	HPI	ROS	PFSH
PF	Brief	None	None
EPF	Brief	≥ 1	None
Detailed	Ext	2 - 9	1/3
Comp	Ext	≥ 10	3/3

Exam	Bullets Required
PF	1 - 5 from any organ systems
EPF	6 - 11 from any organ systems
Detailed	≥ 12 from any organ systems
Comp	2 bullets from NINE systems

MDM	Prob. Pts	Data Pts	Risk
SF	≤ 1	≤ 1	Min
Low	2	2	Low
Mod	3	3	Mod
High	≥ 4	≥ 4	High

Requires 2/3 dimensions

CHIEF COMPLAINT: Blurred vision.

HISTORY OF PRESENT ILLNESS: This is a 77-year-old white male with hypertension. He describes a frontal headache beginning last night. The pain was constant and gnawing. This type of headache is unusual for him. The headache was still present this morning. In addition, he states he felt "fuzzy headed." His vision was somewhat blurred, but there were no symptoms of amaurosis fugax or diplopia. He denies lateralizing weakness or numbness, facial droop or vertigo. He also denies trouble speaking, swallowing or hearing. He decided to drive himself to the post office and then found himself out in the country. Somehow, he drove off into a ditch slowly. There was no damage to the car or injury. A neighbor happened by and picked him up. They called his wife because he seemed shook up and disoriented. She directed them to call an ambulance and the ambulance transported him to the emergency room. Apparently, he was able to ambulate without much difficulty at the scene. His spouse states that his mental functioning seems normal now. The headache is better. He has been taking aspirin daily. His blood sugar this morning was 133.

PAST MEDICAL HISTORY: Coronary artery disease with a history of MI and LBBB Mixed hyperlipidemia. History of tobacco abuse in the form of smoking. He quit in 1998. Prostatism. Hypertension. Adult onset diabetes diagnosed in 2001. Endogenous obesity

MEDICATIONS: (*Assume all meds were listed here.*)

ALLERGIES OR MEDICATION INTOLERANCES: Sulfa, doxycycline, Lipitor and Wellbutrin.

SOCIAL HISTORY: He is married. He denies alcohol consumption and quit smoking over 10 years ago.

FAMILY HISTORY: Positive for hypertension.

REVIEW OF SYSTEMS: Negative for recent significant weight change, diplopia,sudden loss of vision, sudden loss of hearing, shortness of breath, cough, wheezing, chest pain, palpitations, syncope, edema, abdominal pain, dysphagia, constipation, diarrhea, nausea, hematuria, incontinence, dysuria, problematic joints, dizziness, numbness, polyuria and polyphagia.

PHYSICAL EXAMINATION: VITAL SIGNS: Blood pressure 141/74. Pulse 65. O2 saturation is 96% on room air. Respiratory rate is 17. GENERAL: He does not appear acutely or chronically ill. He is alert and his speech is clear. SKIN: Warm and dry without rashes or lesions. HEENT: Sclerae are white. Pupils equal, round, and reactive to light. Extraocular movements are full; however, he has a left homonymous hemianopsia. Hearing is grossly intact. Oropharynx is clear. He is completely edentulous. Tongue protrusion is midline. Palate rises symmetrically. NECK: Supple. No adenopathy, thyromegaly, or jugular venous distention. No carotid bruits. LUNGS: Clear to auscultation and percussion. Respiratory effort is normal. HEART: Regular rate and rhythm with a grade 2/6 systolic ejection murmur. ABDOMEN: Soft and nontender without hepatosplenomegaly or masses. LYMPHATICS: No neck or supraclavicular lymphadenopathy. EXTREMITIES: No clubbing, cyanosis, or edema. No gross joint abnormalities or amputations. NEUROLOGICAL: Sensation is grossly intact to light touch over the face and extremities. Grip is equal bilaterally. No tremors noted. No facial droop. Negative Babinski. Patellar DTRs are equal bilaterally. PSYCHIATRIC: Orientation, memory, mood, affect, and insight appear normal for the situation.

LABORATORY DATA: Chemistry panel shows a sodium of 139, potassium was 4.2, BUN is 21, and creatinine 1.0. Calcium is slightly elevated at 10.4. Transaminases, alkaline phosphatase, and total bilirubin are normal. CBC shows a normal white count of 7.2, hemoglobin was 12.2, and platelet count is 174,000.

DIANOSTIC STUDIES: CT scan of the brain showed some subtle decreased attenuation in the region of the posterior aspect of the temporal lobe suspicious for ischemia. MRI scan shows chronic white matter changes and an old posterior infarct on the left. There were findings of an acute ischemic infarct in the temporal lobe on the right. EKG shows an old left bundle branch block.

IMPRESSION:

1. The patient presents with an acute right temporal lobe ischemic infarct. This is manifest primarily by left visual field loss. He has been taking aspirin. Plavix will be added. Full anticoagulation with Coumadin may be indicated if echocardiogram shows significant left ventricular dysfunction. Carotid Doppler study will be checked and his neurological function will be monitor closely.
2. Coronary artery disease, stable. Continue current medications.
3. Hypertension. Blood pressure is normal. Continue current medications.
4. Adult-onset diabetes mellitus. Current medications will be continued andblood sugar of will be closely.

Problem Points	7
Data Points	5
Risk	

HIGH RISK
ACUTE CVA
"ABRUPT CHANGE IN NEUROLOGICAL STATUS"

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